MID-YEAR CLAIMS · RED FLAG CHECKLIST

Your H1 Data Is Already Talking.

Five claims-data patterns that predict your renewal trajectory before the PBM presents their terms. Run these checks at the H1 mark; the patterns visible at six months become the negotiation in the fall.

01 Five Patterns to Apply

Data pull first: request a 12-month claims extract from your PBM with PMPM trend, specialty as % of total spend, generic fill rate and GER, rebate per claim, and high-cost claimant counts by threshold. Compare H1 against the same period last year. The patterns visible at the mid-year mark are the patterns the PBM has already modeled into their renewal proposal.

Pattern 1 · Specialty Concentration Shift

`Specialty as % of total spend, QoQ`

If specialty as a percentage of total spend is climbing quarter over quarter, high-cost therapies are driving your trend more than utilization volume. Different cost problem; different intervention. New specialty starts, dose escalations, and therapy duration extensions each call for a different response.

- [ ] Track specialty spend as % of total pharmacy spend quarterly
- [ ] Identify which therapeutic categories are driving the shift
- [ ] Compare current ratio to same period last year
- [ ] Determine: new starts, dose escalations, or duration extensions?

Pattern 2 · High-Cost Claimant Clustering

`Members exceeding $50K-$100K thresholds`

One high-cost claimant is an event. Multiple high-cost claimants in the same period is a pattern. Simultaneous threshold crossings compress stop-loss exposure and can trigger specific and aggregate attachment points faster than projected.

- [ ] Count members exceeding $50,000 in claims, by quarter
- [ ] Compare counts to prior periods
- [ ] Count members exceeding $100,000 in claims, by quarter
- [ ] Determine: population health, treatment protocol, or UM gap?

Pattern 3 · Utilization Anomalies

`Volume spikes by drug category vs prior year`

Unexpected volume spikes in specific drug categories can signal formulary gaps, new prescribing trends, or PA criteria that are not working as intended. Controlled substance spikes may indicate UM gaps; specialty spikes may signal new-to-market therapies bypassing pathway review.

- [ ] Compare utilization by drug category vs same period last year
- [ ] Investigate top drugs within flagged categories
- [ ] Flag categories with volume increase exceeding overall trend
- [ ] Check for single-prescriber or single-provider concentration

Pattern 4 · Generic Efficiency Decline

`Generic fill rate + GER, monthly`

Generic fill rate (% prescriptions filled generic) and Generic Effective Rate (% of benchmark price) are independent signals. Declining fill rate suggests brand-to-generic substitution misses; weakening GER suggests pricing erosion even when generics are being dispensed. Both can move at the same time.

- [ ] Track generic fill rate monthly against contract guarantee
- [ ] If fill rate declining: check brand launches without alternatives
- [ ] Track GER monthly against market benchmark
- [ ] If GER weakening: open the pricing conversation pre-renewal

Pattern 5 · Rebate Revenue Erosion

`Rebate per claim, quarterly`

Rebates per claim is more revealing than total rebate dollars. Total rebates can grow because total spend grew. Rebate per claim shows whether PBM rebate efficiency is improving, stable, or declining. A drop may signal formulary mix changes, manufacturer contract renegotiations, or shifts away from rebate-eligible brand products.

- ☐ Track rebate per claim quarterly
- ☐ Determine: PBM-initiated formulary change, manufacturer change, or mix shift?
- ☐ If declining, request explanation from your PBM
- ☐ Document the cause; bring to renewal negotiation

Illustrative pattern checklist for educational purposes. Pattern thresholds should be calibrated to your plan size and prior trend. Two or more patterns flagged simultaneously typically signals a renewal trajectory worth confronting now, not in September.

03 Intervention Pathways by Pattern

Each pattern points to a different intervention. Match the pattern to the action; multiple patterns flagged together usually call for a coordinated response across formulary, contract, and clinical pathways.

PATTERN 1 · SPECIALTY CONCENTRATION SHIFT

Trace the driver before applying interventions.

New specialty starts call for clinical-pathway review and prior authorization tightening. Dose escalations call for utilization management edits and clinical pathway alignment. Therapy duration extensions call for reauthorization protocols. Each requires different conversations with the PBM and the prescribers.

PATTERN 2 · HIGH-COST CLAIMANT CLUSTERING

Coordinate with stop-loss carrier this quarter.

Cluster patterns compress stop-loss exposure. Engage your stop-loss carrier and broker before specific or aggregate attachment points are crossed. Verify high-cost claimants are coded into your stop-loss tracking; review whether disease management programs are activated for the affected members.

PATTERN 3 · UTILIZATION ANOMALIES

Pull the top drugs and trace the cause.

For each flagged category, identify top drugs by spend, check for new-to-market entries, review whether PA criteria changed, and check for prescriber or provider concentration. The cause determines the response: clinical pathway, PA recalibration, or network conversation.

PATTERN 4 · GENERIC EFFICIENCY DECLINE

Open the pricing conversation before renewal.

Declining fill rate is a clinical/operational issue (substitution misses, brand launches without alternatives). Weakening GER is a contract issue (pricing erosion). For GER specifically, the renewal pricing conversation should not wait for the formal PBM presentation.

PATTERN 5 · REBATE REVENUE EROSION

Document the cause; bring to renewal negotiation.

PBM-initiated formulary change is a different conversation than manufacturer contract change or population mix shift. Each has different implications for whether the rebate decline reflects a recoverable structural issue or a market reality. Document the cause so the renewal conversation is informed.

04 Renewal Prep Actions This Quarter

Mid-year is the inflection point. Patterns flagged now have four to five months to be addressed before renewal. The employers who run this checklist mid-year negotiate from a different position than those who wait for the PBM's renewal proposal in September.

§ **Pull the data this month, not next quarter**
Request the H1 claims extract now, even if your fiscal year is calendar-aligned. The patterns are visible by July; waiting until September is the difference between negotiating and reacting.

§ **Document each pattern with the underlying numbers**

Patterns without specific data are talking points. Patterns with PMPM, GER, rebate-per-claim, and threshold counts are negotiating leverage. Build the documentation; bring it to renewal.

§ **Schedule a coordination meeting with broker, stop-loss carrier, PBM**

If two or more patterns are flagged, the renewal conversation needs to be cross-vendor. Schedule the coordination meeting now, with the data in hand. The single best thing your broker can do for you mid-year is run this meeting.

§ **Bring contract gaps to next renewal redline**

Patterns often surface contract gaps: missing performance guarantees on GER, no rebate-per-claim minimum, unclear specialty utilization management. Each pattern flagged is also a redline opportunity at the next contract renewal.